

176mm

187mm

Elimination

No physiological system of elimination exists for iron, and it can accumulate in the body to toxic amounts; however, small amounts are lost daily in the shedding of skin, hair, and nails; and in feces, perspiration, breast milk (0.5 to 1 mg per day), menstrual blood, and urine.

Average daily loss of iron for healthy adults is –

Males: 1 mg per day.

Postmenopausal females: 1 mg per day.

Healthy pre-menopausal adult females: 1.5 to 2 mg per day.

Indications:

Indicated in the prevention and treatment of iron deficiency anemia, which may result from inadequate diet, malabsorption, pregnancy, rapid growth during childhood, and/or blood loss.

Iron deficiency may lead to defective erythropoiesis that may eventually result in a hypochromic anaemia (low haemoglobin concentrations); the red blood cells in iron-deficiency anaemia may be either normocytic (normal size) or microcytic (small in size).

Note: Deficiency of iron may lead to fatigue, shortness of breath, decreased physical performance, impaired learning in children and adults, altered body temperature, and altered immune function.

Contraindications:

Iron compounds should not be given to patients receiving repeated blood transfusions or to patients with anaemias not produced by iron deficiency unless iron deficiency is also present. Oral iron therapy should not be administered concomitantly with parenteral iron.

Except under special circumstances, this medication should not be used when the following medical problems exist:

Hemochromatosis or Hemosiderosis

(Existing iron overload may be increased)

Porphyria cutanea tarda

(May be caused by hepatic accumulation of iron, as in iron overload)

Side Effects / Adverse Reactions:

Those indicating need for medical attention

Incidence more frequent

Abdominal or stomach pain, cramping, or soreness

Incidence less frequent or rare

Contact irritation (chest or throat pain, especially when swallowing; stools containing fresh or digested blood)

Those indicating need for medical attention only if they continue or are bothersome

Incidence more frequent

Constipation; diarrhoea; nausea; vomiting

Incidence less frequent

Darkened urine (iron sulfide formation following large doses); *heartburn*;

Precautions / Warnings:

Care should be taken when given to patients with iron-storage or iron-absorption diseases, haemoglobinopathies, or existing gastrointestinal disease.

Pregnancy –

In the first trimester of pregnancy, adequate iron intake is usually obtained from a proper diet; however, in the second and third trimesters, when iron deficiency is more prevalent because of greatly increased requirements, iron supplements may be recommended. However, some clinicians prefer to evaluate the patient before giving routine iron supplementation.

Studies in humans have not been done, and problems in humans have not been documented with intake of normal daily recommended amounts.

Breast Feeding

Problems in humans have not been documented with intake of normal daily recommended amounts.

Geriatrics

Problems in geriatrics have not been documented with intake of normal daily recommended amounts. Some geriatric patients may require a larger than usual daily ingestion of bioavailable iron to correct an iron deficiency, because their ability to absorb iron has been diminished by reduced gastric secretions and achlorhydria.

Drug Interactions

Antacids or Calcium supplements (calcium carbonate or phosphate) or **Coffee or Eggs or Foods or medications containing bicarbonates, carbonates, oxalates, or phosphates or Milk or milk products or Tea or Whole-grain breads and cereals (contain phytic acid) and dietary fiber**

(Concurrent use with iron may decrease iron absorption because of the formation of less soluble or insoluble complexes; iron supplements should not be taken within 1 hour before or 2 hours after ingestion of any of the above)

Fluoroquinolones

(Iron may reduce absorption of fluoroquinolones by chelation; fluoroquinolones should be taken at least 2 hours before or 2 hours after iron supplements)

Penicillamine or Trientine

(Concurrent use with iron supplements may decrease the therapeutic effects of these medications; a period of 2 hours should elapse between administration of penicillamine or trientine and iron)

Tetracyclines, oral

(Concurrent use with iron reduces absorbability of oral tetracyclines; patients should be advised to take iron supplements 2 hours after tetracycline)

Recommended Dosage, Dosage Schedule and Route of Administration:

Usual adult dose:

i. Prevention of iron-deficiency anaemia: 1 tablet daily.

ii. Treatment of iron-deficiency anaemia: 1 to 2 tablets daily in divided doses.

Children (less than 12 years old): Not recommended.



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User Instructions:

Take on empty stomach 1 hour before or 2 hours after meals; or with food to lessen possibility of stomach upset.
Take with water or fruit juice, a full glass (240 mL) for adults.

Symptoms and Treatment for Overdose and Antidote (s):

Symptoms

Early symptoms of acute iron toxicity.

Diarrhoea, sometimes containing blood; fever, nausea, severe stomach pain or cramping, sharp; vomiting, severe, sometimes containing blood

Note: Early symptoms may not be evident for up to 60 minutes or longer; if overdose is suspected, emergency room treatment should not be delayed for evidence of symptoms, but should begin immediately.

A latency period lasting from 2 to about 48 hours after ingestion may occur between the 2 symptomatic phases. During this time, the patient may appear to improve clinically.

Late symptoms of acute iron toxicity

Bluish-coloured lips, fingernails, palms of hands; drowsiness; pale, clammy skin; seizures; unusual tiredness or weakness; weak and fast heartbeat.

Treatment

Overdose of ingested iron can be fatal, especially in small children.

Immediate treatment is essential. Serious poisoning may result in small children from ingestion of 200 mg of elemental iron.

In treating acute iron poisoning, speed is essential to block absorption of iron from the alimentary tract. Emesis or lavage should be considered and serum-iron concentrations may be an aid to estimating the severity of poisoning. Chelation therapy with desferrioxamine may be necessary.

Other measures include the symptomatic management and therapy of metabolic and cardiovascular disorders.

Packing / Pack Sizes:

Blister of 10's, 10x10's/box, 100x10's/box.

Storage Conditions, User Instructions, and Pharmaceutical Precautions:

Store below 30°C in a tight container.

Shelf Life: 4 years

MANUFACTURER AND PRODUCT REGISTRATION HOLDER:

SM PHARMACEUTICALS SDN BHD (218620-M)
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SM PHARMACEUTICALS SDN BHD

FERROMED (FERROUS FUMARATE) TABLET 200 MG

Description:

A brown, round, biconvex, film coated tablet with break-line on one side.

Composition:

Each tablet contains: Ferrous Fumarate 200 mg

Actions and Mode or Mechanisms of Action:

Iron is an essential component of hemoglobin, adequate amounts of which are necessary for effective erythropoiesis and the resultant oxygen transport capacity of the blood. A similar function is provided by iron in myoglobin production. Iron also serves as a cofactor of several essential enzymes, including cytochromes that are involved in electron transport. Iron is necessary for catecholamine metabolism and the proper functioning of neutrophils.

Pharmacology (Summary of Pharmacodynamics and Pharmacokinetics):

Absorption

Absorption is increased when iron stores are depleted or red blood cell production is increased. Conversely, high iron blood concentrations decrease absorption.

Iron-deficient individuals: 10 to 30% is absorbed.

Non – iron-deficient individuals: Approximately 5 to 15% of ingested iron is absorbed.

Absorption occurs principally in the duodenum and proximal jejunum.

Absorption is most efficient when iron is ingested in its ferrous rather than its ferric form, on an empty stomach. Gastric acid increases absorption by maintaining ferric iron in a soluble form.

Twenty to 30% of heme iron is absorbed from the diet. Two to 10% of non-heme iron is absorbed from the diet, and its absorption is affected by other foods ingested. Ascorbic acid enhances the absorption of non-heme iron. Meat and other animal tissues also enhance the absorption of non-heme iron.

Distribution

Iron is transported in the body to bone marrow for red blood cell production in the iron-transferrin complex form.

Protein binding

Very high (90% or more).

Hemoglobin – High.

Myoglobin, enzymes, and transferrin – Low.

Ferritin and hemosiderin – Low.

Storage

Iron is stored as ferritin and hemosiderin, primarily in hepatocytes and in the reticuloendothelial system, with some storage in muscle.

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